

1 ENGROSSED HOUSE
2 BILL NO. 3650

By: Stinson of the House

3 and

4 Rosino of the Senate
5

6 An Act relating to poor persons; amending 56 O.S.
7 2021, Section 4002.12, as last amended by Section 7,
8 Chapter 448, O.S.L. 2024 (56 O.S. Supp. 2025, Section
9 4002.12), which relates to minimum rates of
10 reimbursement; extending deadlines; and providing an
11 effective date.

12 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

13 SECTION 1. AMENDATORY 56 O.S. 2021, Section 4002.12, as
14 last amended by Section 7, Chapter 448, O.S.L. 2024 (56 O.S. Supp.
15 2025, Section 4002.12), is amended to read as follows:

16 Section 4002.12. A. Until July 1, ~~2027~~ 2028, the Oklahoma
17 Health Care Authority shall establish minimum rates of reimbursement
18 from contracted entities to providers who elect not to enter into
19 value-based payment arrangements under subsection B of this section
20 or other alternative payment agreements for health care items and
21 services furnished by such providers to enrollees of the state
22 Medicaid program. Except as provided by subsection I of this
23 section, until July 1, ~~2027~~ 2028, such reimbursement rates shall be
24 equal to or greater than:

1 1. For an item or service provided by a participating provider
2 who is in the network of the contracted entity, one hundred percent
3 (100%) of the reimbursement rate for the applicable service in the
4 applicable fee schedule of the Authority; or

5 2. For an item or service provided by a non-participating
6 provider or a provider who is not in the network of the contracted
7 entity, ninety percent (90%) of the reimbursement rate for the
8 applicable service in the applicable fee schedule of the Authority
9 as of January 1, 2021.

10 B. A contracted entity shall offer value-based payment
11 arrangements to all providers in its network capable of entering
12 into value-based payment arrangements. Such arrangements shall be
13 optional for the provider but shall be tied to reimbursement
14 incentives when quality metrics are met. The quality measures used
15 by a contracted entity to determine reimbursement amounts to
16 providers in value-based payment arrangements shall align with the
17 quality measures of the Authority for contracted entities.

18 C. Notwithstanding any other provision of this section, the
19 Authority shall comply with payment methodologies required by
20 federal law or regulation for specific types of providers including,
21 but not limited to, Federally Qualified Health Centers, rural health
22 clinics, pharmacies, Indian Health Care Providers and emergency
23 services.

1 D. A contracted entity shall offer all rural health clinics
2 (RHCs) contracts that reimburse RHCs using the methodology in place
3 for each specific RHC prior to January 1, 2023, including any and
4 all annual rate updates. The contracted entity shall comply with
5 all federal program rules and requirements, and the transformed
6 Medicaid delivery system shall not interfere with the program as
7 designed.

8 E. The Oklahoma Health Care Authority shall establish minimum
9 rates of reimbursement from contracted entities to Certified
10 Community Behavioral Health Clinic (CCBHC) providers who elect
11 alternative payment arrangements equal to the prospective payment
12 system rate under the Medicaid State Plan.

13 F. The Authority shall establish an incentive payment under the
14 Supplemental Hospital Offset Payment Program that is determined by
15 value-based outcomes for providers other than hospitals.

16 G. Psychologist reimbursement shall reflect outcomes.
17 Reimbursement shall not be limited to therapy and shall include but
18 not be limited to testing and assessment.

19 H. Coverage for Medicaid ground transportation services by
20 licensed Oklahoma emergency medical services shall be reimbursed at
21 no less than the published Medicaid rates as set by the Authority.
22 All currently published Medicaid Healthcare Common Procedure Coding
23 System (HCPCS) codes paid by the Authority shall continue to be paid
24 by the contracted entity. The contracted entity shall comply with

1 all reimbursement policies established by the Authority for the
2 ambulance providers. Contracted entities shall accept the modifiers
3 established by the Centers for Medicare and Medicaid Services
4 currently in use by Medicare at the time of the transport of a
5 member that is dually eligible for Medicare and Medicaid.

6 I. 1. The rate paid to participating pharmacy providers is
7 independent of subsection A of this section and shall be the same as
8 the fee-for-service rate employed by the Authority for the Medicaid
9 program as stated in the payment methodology in OAC 317:30-5-78,
10 unless the participating pharmacy provider elects to enter into
11 other alternative payment agreements.

12 2. A pharmacy or pharmacist shall receive direct payment or
13 reimbursement from the Authority or contracted entity when providing
14 a health care service to the Medicaid member at a rate no less than
15 that of other health care providers for providing the same service.

16 J. Notwithstanding any other provision of this section,
17 anesthesia shall continue to be reimbursed equal to or greater than
18 the anesthesia fee schedule established by the Authority as of
19 January 1, 2021. Anesthesia providers may also enter into value-
20 based payment arrangements under this section or alternative payment
21 arrangements for services furnished to Medicaid members.

22 K. The Authority shall specify in the requests for proposals a
23 reasonable time frame in which a contracted entity shall have
24

1 entered into a certain percentage, as determined by the Authority,
2 of value-based contracts with providers.

3 L. Capitation rates established by the Oklahoma Health Care
4 Authority and paid to contracted entities under capitated contracts
5 shall be updated annually and in accordance with 42 C.F.R., Section
6 438.3. Capitation rates shall be approved as actuarially sound as
7 determined by the Centers for Medicare and Medicaid Services in
8 accordance with 42 C.F.R., Section 438.4 and the following:

9 1. Actuarial calculations must include utilization and
10 expenditure assumptions consistent with industry and local
11 standards; and

12 2. Capitation rates shall be risk-adjusted and shall include a
13 portion that is at risk for achievement of quality and outcomes
14 measures.

15 M. The Authority may establish a symmetric risk corridor for
16 contracted entities.

17 N. The Authority shall establish a process for annual recovery
18 of funds from, or assessment of penalties on, contracted entities
19 that do not meet the medical loss ratio standards stipulated in
20 Section 4002.5 of this title.

21 O. 1. The Authority shall, through the financial reporting
22 required under subsection G of Section 4002.12b of this title,
23 determine the percentage of health care expenses by each contracted
24 entity on primary care services.

2. Not later than the end of the fourth year of the initial contracting period, each contracted entity shall be currently spending not less than eleven percent (11%) of its total health care expenses on primary care services.

3. The Authority shall monitor the primary care spending of each contracted entity and require each contracted entity to maintain the level of spending on primary care services stipulated in paragraph 2 of this subsection.

SECTION 2. This act shall become effective November 1, 2026.

Passed the House of Representatives the 11th day of March, 2026.

Presiding Officer of the House
of Representatives

Passed the Senate the _____ day of _____, 2026.

Presiding Officer of the Senate